Topical and Systemic Treatment Approaches

Topical measures are aimed at supportive care and minimizing exacerbating factors. Systemic treatment includes prednisone, colchicine, and thalidomide.

Behçet Disease

Behçet disease (see Chap. 167), a rare multisystem disorder most prevalent in Japan, Turkey, and the Middle East, is characterized by oral and genital ulcers, iridocyclitis, and variable presence of synovitis, meningoencephalitis, and cutaneous pustular vasculitis with aphthae. At least two of the other findings are required for diagnosis. The oral ulcerations of Behçet disease are an almost universal finding and often herald the onset of this syndrome. The lesions present as multiple aphthous-like ulcers that heal within a few weeks and recur at least three times per year (Fig. 74-9).

Cyclic Neutropenia

Cyclic neutropenia, an autosomal dominant disorder characterized by cyclic decreases in the number of circulating neutrophils, presents with ulcerative gingivostomatitis that lasts a few days and recurs at intervals of 21 to 27 days. Progression to periodontal disease with subsequent alveolar bone loss is seen over time. Spontaneous improvement is seen as the neutrophil count recovers, but systemic antibiotic therapy may be required to prevent bone loss.

Non-Aphthous Erosions and Ulcers

Shallow, painful erosions on the gingiva and buccal mucosa, which at times may be extensive and may lead to ulcerations, occur in pemphigus vulgaris (see Chap. 52; Fig. 74-10), paraneoplastic pemphigus (see Chap. 53), and, less frequently, in bullous pemphigoid (see Chap. 54).

Virtually all patients with cicatricial pemphigoid (see Chap. 55) develop mucosal lesions that consist of a desquamative gingivitis with painful erosions on the tongue, buccal and palatal mucosa, and ocular symblepharon.

Oral erosions that heal without scarring are commonly seen in both epidermolysis bullosa (EB) simplex and junctional EB (see Chap. 60). Patients with dystrophic EB demonstrate the most severe oral involvement. In the recessive form, caries, diffuse oral ulceration, scarring, microstomia, vestibular obliteration, and ankyloglossia are frequent findings. The dominant form has similar features but demonstrates less severity (see Chap. 60). Minimizing trauma and optimizing nutrition are paramount, and early referral to dentistry is key in maintaining oral function.

TABLE 74-5: Systemic Conditions Associated with Recurrent Oral Ulcers

- Hematinic deficiency (up to 20%)
- Gastrointestinal malabsorption (3%)
- Celiac disease, dermatitis herpetiformis, and gluten-sensitive enteropathy (HLA DRW10 and DQW1)
- Crohn disease
- Pernicious anemia
- Systemic lupus erythematosus
- Human immunodeficiency virus
- Behçet disease
- Trisomy 8, myelodysplasia
- Cyclic neutropenia
- PFAPA (periodic fever, aphthous stomatitis, pharyngitis, and cervical adenitis)
- MAGIC (mouth and genital ulcers with inflamed cartilage)

TABLE 74-6: Clinical and Laboratory Workup of Recurrent Oral Ulcers

No specific tests available; focus is on excluding other disorders.

- Obtain a complete medical history and review of systems
- Comprehensive hematologic evaluation
- Complete blood cell count
- Erythrocyte sedimentation rate
- Serum iron, ferritin levels, total iron binding capacity
- Folate, vitamin B_{12}, vitamin B_1, vitamin B_2, vitamin B_6
- Weekly complete blood cell count $\times$ 5
- Serum anti-endomysium antibody and transglutaminase assay (positive in celiac disease)
- Biopsy for hematoxylin and eosin with special stains
- Tzanck smear, viral studies, and viral cultures
- Colonoscopy, especially in the setting of gastrointestinal complaints
- Central nervous system evaluations, including examination and magnetic resonance imaging

All positive findings should be addressed.